



“The narrative that private equity increases costs and reduces quality is wrong. They want the best outcomes. They focus on governance, processes”

cities and some in Tier-III cities. It will be a gradual progression. As we move into newer towns outside the capitals, we will naturally look at smaller towns as well.

Cancer is also a big area of focus for us. Pretty much all our new hospitals are going to have a bunker and linear accelerators.

Q: The IPO was subscribed nearly five times and listed at a premium. How do you justify Manipal Hospitals' valuation?

A: The 85-times P/E multiple has been projected a little unfairly. Yes, it is 85 times, but that is because of the debt we had after acquiring Sahyadri. The right way to look at it is EV/EBITDA. On that basis, we are lower than some of our peers.

We did that deliberately because we wanted to leave some money on the table for investors. The money raised will be used to pay down the debt. The P/E will come down. That is how people should evaluate us post-IPO. It is unfair to look only at the P/E multiple.

Q: Do you think the IPO would have gone as well if the market had priced you on EV/EBITDA from Day One instead of the P/E multiple? How do you plan to deploy the proceeds?

A: I believe so. The fundamentals do not change depending on which multiple you look at first. Investors who did their homework were already looking at it that way. About ₹5,500 crore will be used to pay down the debt we took for Sahyadri, and another ₹500-odd crore will be used to buy the remaining 10% stake from the shareholder. We will keep the rest for future acquisitions. We also have a lot of greenfield hospitals coming up.

Q: As India expands its healthcare infrastructure, what policy reforms are needed to improve access to quality healthcare, and where do you see the biggest opportunity in health insurance?

A: Health insurance penetration needs to keep increasing. Healthcare is expensive. Input costs, whether it is drugs or cutting-edge medical equipment, continue to rise. Healthcare will continue to become more expensive. That said, it is still a fraction of the cost of the Western world. But if people cannot afford it (even ₹50,000), it is difficult for them to come to a hospital. The government health insurance should probably be expanded, and more people should take private health insurance because it is not too expensive. **BT**

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